



TM

HINAPHARM

REG. NO.: MAL
CONTROLLED MEDICINES
UBAT TERKAWAL
FOR ANIMAL USE ONLY

HINAFEN

5% W/W GRANULES

Keep in cool and dry place under 25°C

Keep out of reach of children
Jauhi daripada kanak-kanak

Manufactured by:
HINAPHARM PHARMACEUTICAL CO.,
LTD. FOSHAN
Shashuihe West Road,
Cangjiang Industrial Zone,
Gaoming District, Foshan, Guangdong.

Registration Holder:
Gladron Chemicals Sdn Bhd
No. 7, Jalan TP 7,
UEP Industrial Park,
40400 Shah Alam,
Selangor Darul Ehsan, Malaysia.

20kg

Date of revision: 210513

[Product Name]: Hinafen 5% w/w Granules
[Composition]: Granulated Fenbendazole Premix 5% (w/w)
[Batch No.]:
[Manufacture Date]:
[Expiry Date]:
[Product Description]: Almost white granules.
[Dosage and administration]: When incorporated at a rate of below 2kg/tonne of final feed, the product must only be mixed by a manufacturer who is approved to mix at this level. No dietary controls are required before or after treatment.
>Pigs: The product is suitable for herd medication of pigs. The standard dose rate is 5mg fenbendazole per kg bodyweight. - Routine treatment - herd medication: May be administered to pigs either as a single dose or by divided dosage over 7 days. It may also be administered to sows over 14 days. All methods of treatment are equally effective. To avoid underdosing, at incorporation rate, calculations should be based on the heaviest pig in the group. - Treatment for specific infections: for the treatment of *Metastrongylus apri* (lung worm) and/or *Trichuris suis*, it is recommended that the dosage is divided and administered over 7 days for most effective treatment. - Suggested dosing programmes for pigs: - Sows and breeders: Sows before entering farrowing accommodation and again after weaning, this will maintain the sows' condition and reduce the risk of infection to her piglets. Sows should be treated every 6 months. - Growing and finishing pigs: Sows sows on purchase or on moving onto finishing houses. Where there is a high risk of Ascaridosis ("Milk Spot Liver"), repeat treatment at monthly intervals. Whole herd treatment: Treatment of the whole herd (all adult and young stock) simultaneously every 6 months and minimize the risk of reinfection. This should be carried out routinely unless recommended otherwise by a veterinary surgeon. It should also be combined with careful attention to environmental hygiene which is especially important for control of "Milk Spot Liver". - Outdoor pigs: When clean grazing is available, dose before moving stock onto it. If clean land is not available then pigs should be treated monthly from July to October.
>Cattle: Cattle can be treated by dividing the dose over 5 or 10 consecutive days.
a) 5-day treatment: - Administer a daily dose of 1.5mg of fenbendazole per kg bodyweight for 5 consecutive days.
b) 10-day treatment: - Administer a daily dose of 0.75mg fenbendazole per kg bodyweight for 10 consecutive days. This is most easily achieved by mixing 15g product with 100g feed and feeding 1kg medicated feed per 100kg bodyweight for 5 days.
>Horses: Administer orally 7.5mg fenbendazole/kg bodyweight, twice daily course. - For the treatment and control of migrating larval stages of large strongyles and encysted mucosal 3rd and 4th stage larvae and inhibited 3rd stage small strongyle larvae (encysted) in the mucosa, administer 7.5mg fenbendazole/kg bodyweight daily for 5 days. - Single dose treatments: - For the treatment and control of encysted mucosal stages of small strongyles, administer 30mg fenbendazole/kg bodyweight. - For the treatment and control of migrating stages of large strongyles, administer 60mg fenbendazole/kg bodyweight. - Diarrhoea caused by Strongylidae is often in two to three week old sucking foals should be treated with 50mg fenbendazole/kg bodyweight. - To ensure administration of a correct dose, bodyweight should be determined as accurately as possible. Accuracy of the dosing device should be checked. - Recommended dosing programme: - All horses should be routinely wormed with the single dose of product. - Treatment of encysted mucosal dwelling larvae should ideally be done in the autumn (late October/November) and again in the Spring. - However, for horses who fail to maintain condition or bought in horses with unknown worming history, treatment can be given at any time of the year.
[Warnings/Precautions]:
i) Special precautions for use in animals:
a) Cattle: The product may be effective against *Trichuris* and Strongylidae species when given by divided dosage in cattle.
>Pigs: When preparing medicated feed for pigs which are to be administered over a number of days, to ensure thorough dispersion a premix should be prepared first. Once mixed, this should then be incorporated with the remaining feed. The product can be incorporated into pelleted feed preconditioned with steam at a temperature not greater than 45°C. Only to be incorporated by a manufacturer whose approval to mix at a rate below 2kg per tonne of final feed.
>Horses: Do not use in horses and other equines intended for human consumption. Resistance to fenbendazole has been reported in cyathostomes in horses. Therefore the use of this product should be based on local (regional, farm) epidemiological information about susceptibility of nematodes and recommendations on how to limit further selection for resistance to anthelmintics.
>Care should be taken to avoid the following practices because they increase the risk of development of resistance and could ultimately result in ineffective therapy. Too frequent and repeated use of anthelmintics from the same class, over an extended period of time. Under-dosing which may be due to underestimation of bodyweight, misadministration of the product, or lack of calibration of the dosing device (if any). Suspected clinical cases of resistance to anthelmintics should be further investigated using appropriate tests (eg. Faecal Egg Count Reduction Test) where the results of the test(s) strongly suggest resistance to a particular anthelmintic and anthelmintic belonging to another pharmacological class and having a different mode of action should be used.
ii) Special precautions to be taken by the person administering the medicinal product to the animals:
- Direct contact with the skin should be kept to a minimum. Avoid inhalation of granule dust. Wash hands after use.
[Withdrawal Period]: The product is not intended to be used in horses for human consumption. Animals must not be slaughtered for human consumption during treatment.
[Pharmacodynamics]: Fenbendazole is an anthelmintic belonging to the benzimidazole-carbamate group. It acts by interfering with the energy metabolism of the nematode.
[Pharmacokinetics]: Fenbendazole inhibits the polymerization of tubulin to microtubules. Fenbendazole is effective against adult and immature stages.
[Pharmacokinetics]:
>Pigs and cattle: Fenbendazole is only partly absorbed after oral administration and is then metabolized in the liver. The half-life of fenbendazole in serum after oral application of the recommended dose in cattle is 10-18 hours and in sheep 21-33 hours. Fenbendazole and its metabolites are distributed throughout the body and high concentrations can be found in the liver. The elimination of fenbendazole and its metabolites occurs primarily via the faeces (>90%) and to a small extent in the urine and milk. Fenbendazole is metabolized to 5-sulphoxide then to sulphone and amines.
>Horses: Fenbendazole is only partly absorbed from the intestine and reaches maximum plasma concentration 6 (4-8) hours after oral administration. Fenbendazole is metabolised mainly by enzymes of the cytochrome P-450 system in the liver. The major oxidative metabolite is fenbendazole sulfoxide which is further metabolized to fenbendazole sulfone. Fenbendazole and its metabolites are distributed throughout the body at highest concentrations are found in the liver. Fenbendazole and its metabolites are detectable in the plasma only during the first 48 hours following drug administration at a single dose rate of 10 mg fenbendazole/kg bodyweight. Administration of fenbendazole at a dose rate of 10mg/kg bodyweight daily for five consecutive days lead to accumulation of fenbendazole during the multiple dosing period whereas the concentrations of the two metabolites show only a slight increase. After the last administration on day 5, all three compounds are eliminated from blood very rapidly, within two or three days. The elimination of fenbendazole and its metabolites occurs primarily via the faeces.
[Indications]: Pigs: A broad spectrum anthelmintic for the treatment of pigs infected with immature and mature forms of gastro-intestinal and respiratory nematodes, including: *Metastrongylus rubeus* (red stomach worm), *Oesophagostomum* spp. (nodular worm), *Ascaris suum* (round worm), *Trichuris suis* (whipworm), *Metastrongylus apri* (lung worm) (as an aid in its control). Cattle: For the treatment of cattle infected with mature and developing immature forms of nematodes of the gastro-intestinal and respiratory tracts, including: *Ostophorus* spp., *Haemonchus* spp., *Oesophagostomum* spp., *Ostophorus* spp., *Trichostrongylus* spp., *Strongylus* spp., *Bunostomum* spp., *Tricurus* spp., *Nematodirus* spp., *Dicrocoelium* spp. Horses: For the treatment and control of adult and immature round worms of the gastrointestinal tract in horses and other equines. - The product effectively treats and controls the following round worm infections: Large strongyles (adults and migrating larval stages of *S. vulgaris*; adults and tissue larval stages of *S. edentatus*); - Fenbendazole is also effective against small strongyles (*Cyathostomes*). - The product is also effective against encysted inhibited 3rd stage small strongyle larvae in the mucosa. - Adult and immature *Oxyuris* spp., *Strongylus* spp. and *Parascaris equorum*. - Fenbendazole also has an ovicidal effect on nematode eggs.
[Contraindications]: None known.
[Pregnancy and lactation]: Fenbendazole is considered safe to use in pregnant bitches and is generally considered safe to use in pregnancy in all species.
[Interactions with other Medicaments]: None known.
[Adverse Effects]: None known.
[Overdose and Treatment]: None known.